Palbociclib with endocrine therapy

The usual palbociclib starting dose is 125 mg once daily; a lower dose may be prescribed. Palbociclib is taken once daily on Days 1–21, then paused on Days 22–28 of each 28-day cycle. The aromatase inhibitor continues daily, or fulvestrant follows its injection schedule. Ovarian suppression has its own plan when required.

Make treatment fit into daily life

Check whether the pharmacy supplied capsules or tablets: capsules are taken with food; tablets can be taken with or without food. Swallow whole at a consistent time. Avoid grapefruit. If you miss or vomit a dose, do not take an extra dose; take the next scheduled dose. Do not restart after a clinical hold until the treating team confirms.

Before starting

Confirm the exact medicine, strength, endocrine partner, baseline tests and first review appointments. Ask who arranges tests and who reviews the results.

Taking treatment

Check whether the pharmacy supplied capsules or tablets: capsules are taken with food; tablets can be taken with or without food. Swallow whole at a consistent time. Avoid grapefruit. If you miss or vomit a dose, do not take an extra dose; take the next scheduled dose. Do not restart after a clinical hold until the treating team confirms.

Your endocrine partner has a separate timetable

An aromatase inhibitor is normally taken daily through the targeted-medicine rest week. If fulvestrant is the chosen partner, the usual schedule is 500 mg by intramuscular injection on Days 1, 15 and 29, then once monthly; confirm the prescribed dose and appointment dates. Any ovarian-suppression injections have their own schedule.

Treatment duration

Repeat while beneficial and tolerated. The planned week off concerns palbociclib, not automatically the endocrine partner.

Supply and appointments

Request the next supply before tablets run out. Blood tests and any electrocardiogram (ECG) or injections need their own appointments. This oral treatment does not itself require a central line or infusion pump.

If treatment is held

Ask which medicine is held, whether the endocrine partner continues, what needs to recover, when the next review is due and who will authorise restart.

Support between appointments

Keep the oncology contact details and prescribed symptom medicines accessible. New severe symptoms need prompt assessment rather than waiting for the next blood test.

Who arranges each check?

Agree the due window, booking team, result reviewer and whom to contact if an appointment or result is missing. This reference page does not book or record a test.

Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.

- NCCP 00414 v5a · official regimen
- EMA Ibrance SmPC / patient leaflet; product information updated 7 March 2025, sections 4.1–4.8 and 5.1
- Pan-Asian adapted ESMO early breast cancer guidelines
- Pan-Asian adapted ESMO metastatic breast cancer guidelines
- ESO-ESMO ABC 5 consensus: tumour-marker interpretation
- PALOMA-2, palbociclib + letrozole: EMA efficacy tables 6 and 8
- PALOMA-3, palbociclib + fulvestrant: EMA efficacy table 9
- PALOMA-2 primary report, Finn et al 2016
- PALOMA-2 final OS, Slamon et al 2024
- PALOMA-3 protocol-specified final OS, Turner et al 2018
- PALOMA-3 exploratory extended follow-up, Cristofanilli et al 2022

Source reconciled; independent clinical, pharmacy and patient/PPI review pending